

DIAGNOSTIC REPORT



PATIENT NAME : ANGEL JOSEPH

REF. DOCTOR : DR. DR MEENA

ANGEL JOSEPH
B-404, GOODWILL GARDEN, SEC 8, KHARGHAR
9870331316

ACCESSION NO : 0040YL001492
PATIENT ID : ANGEF16030940
CLIENT PATIENT ID:
ABHA NO :

AGE/SEX : 16 Years Female
DRAWN : 07/12/2025 07:23:01
RECEIVED : 07/12/2025 07:27:51
REPORTED : 07/12/2025 12:13:54

Test Report Status	Final	Results	Biological Reference Interval	Units
--------------------	-------	---------	-------------------------------	-------

BIOCHEMISTRY

LIVER FUNCTION PROFILE, SERUM

BILIRUBIN, TOTAL	0.21 Low	0.3 - 1.2	mg/dL
METHOD : COLORIMETRIC DIAZO METHOD			
BILIRUBIN, DIRECT	0.11	< or = 0.3	mg/dL
METHOD : DIAZO METHOD			
BILIRUBIN, INDIRECT	0.10	0.1 - 1.0	mg/dL
METHOD : CALCULATED PARAMETER			
TOTAL PROTEIN	7.1	6.0 - 8.0	g/dL
METHOD : BIURET - COLORIMETRIC ASSAY			
ALBUMIN	4.6 High	3.2 - 4.5	g/dL
METHOD : BROMOCRESOL GREEN			
GLOBULIN	2.5	2.0 - 3.5	g/dL
ALBUMIN/GLOBULIN RATIO	1.8	1.0 - 2.1	RATIO
ASPARTATE AMINOTRANSFERASE(AST/SGOT)	16	< OR = 35	U/L
METHOD : UV WITHOUT PYRIDOXAL-5 PHOSPHATE			
ALANINE AMINOTRANSFERASE (ALT/SGPT)	15	< OR = 35	U/L
METHOD : UV WITHOUT PYRIDOXAL-5 PHOSPHATE			
ALKALINE PHOSPHATASE	67	50 - 117	U/L
METHOD : PNPP, AMP BUFFER-IFCC			
GAMMA GLUTAMYL TRANSFERASE (GGT)	12	0 - 40	U/L
METHOD : G-GLUTAMYL-CARBOXY-NITROANILIDE-IFCC			
LACTATE DEHYDROGENASE	158	125 - 220	U/L
METHOD : LACTATE -PYRUVATE			

LIPID PROFILE PLUS

GLUCOSE FASTING (FBS), FLUORIDE PLASMA

FBS-FASTING BLOOD SUGAR(GLUCOSE)	82	(Normal <100, Impaired fasting glucose: 100 to 125, Diabetes mellitus: >=126 (on more than 1 occasion) (ADA guidelines 2024))
METHOD : HEXOKINASE		

Dr. Archa Rajagopal
Consultant Pathologist

Dr. Mohsin Kamaal
Consultant Pathologist

Page 1 Of 5



View Details



View Report

PERFORMED AT :

Agilus Diagnostics Ltd
Bhoomi Tower, 1st Floor, Hall No.1, Plot No.28 Sector 4, Kharghar
Navi Mumbai, 410210
Maharashtra, India
Tel : 9111591115, Fax :
CIN - U74899PB1995PLC045956



ULR No. 775000015428602-0040

DIAGNOSTIC REPORT



PATIENT NAME : ANGEL JOSEPH

REF. DOCTOR : DR. DR MEENA

ANGEL JOSEPH
B-404, GOODWILL GARDEN, SEC 8, KHARGHAR

9870331316

ACCESSION NO : **0040YL001492**
PATIENT ID : ANGEF16030940
CLIENT PATIENT ID :
ABHA NO :

AGE/SEX : 16 Years Female
DRAWN : 07/12/2025 07:23:01
RECEIVED : 07/12/2025 07:27:51
REPORTED : 07/12/2025 12:13:54

Test Report Status	Results	Biological Reference Interval	Units
--------------------	---------	-------------------------------	-------

Interpretation(s)

LIVER FUNCTION PROFILE, SERUM-

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Bilirubin is excreted in bile and urine, and elevated levels may give yellow discoloration in jaundice. **Elevated levels** results from increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in Viral hepatitis, Drug reactions, Alcoholic liver disease. Conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts, tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of Hemolytic or pernicious anemia, Transfusion reaction & a common metabolic condition termed Gilbert syndrome, due to low levels of the enzyme that attaches sugar molecules to bilirubin.

AST is an enzyme found in various parts of the body. AST is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health. AST levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. AST levels may also increase after a heart attack or strenuous activity. ALT test measures the amount of this enzyme in the blood. ALT is found mainly in the liver, but also in smaller amounts in the kidneys, heart, muscles, and pancreas. It is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health. AST levels increase during acute hepatitis, sometimes due to a viral infection, ischemia to the liver, chronic hepatitis, obstruction of bile ducts, cirrhosis.

ALP is a protein found in almost all body tissues. Tissues with higher amounts of ALP include the liver, bile ducts and bone. Elevated ALP levels are seen in Biliary obstruction, Osteoblastic bone tumors, osteomalacia, hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Pagets disease, Rickets, Sarcoidosis etc. Lower-than-normal ALP levels seen in Hypophosphatasia, Malnutrition, Protein deficiency, Wilsons disease.

GGT is an enzyme found in cell membranes of many tissues mainly in the liver, kidney and pancreas. It is also found in other tissues including intestine, spleen, heart, brain and seminal vesicles. The highest concentration is in the kidney, but the liver is considered the source of normal enzyme activity. Serum GGT has been widely used as an index of liver dysfunction. Elevated serum GGT activity can be found in diseases of the liver, biliary system and pancreas. Conditions that increase serum GGT are obstructive liver disease, high alcohol consumption and use of enzyme-inducing drugs etc.

Total Protein also known as total protein, is a biochemical test for measuring the total amount of protein in serum. Protein in the plasma is made up of albumin and globulin. Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma, Waldenstroms disease. Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic syndrome, Protein-losing enteropathy etc.

Albumin is the most abundant protein in human blood plasma. It is produced in the liver. Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting etc.

GLUCOSE FASTING (FBS), FLUORIDE PLASMA-TEST DESCRIPTION

Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and so that no glucose is excreted in the urine.

Increased in: Diabetes mellitus, Cushing's syndrome (10 - 15%), chronic pancreatitis (30%). Drugs: corticosteroids, phenytoin, estrogen, thiazides.

Decreased in: Pancreatic islet cell disease with increased insulin, insulinoma, adrenocortical insufficiency, hypopituitarism, diffuse liver disease, malignancy (adrenocortical, stomach, fibrosarcoma), infant of a diabetic mother, enzyme deficiency diseases (e.g. galactosemia), Drugs-insulin, ethanol, propranolol, sulfonylureas, tolbutamide, and other oral hypoglycemic agents.

NOTE: While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values), there is wide fluctuation within individuals. Thus, glycosylated hemoglobin (HbA1c) levels are favored to monitor glycemic control.

High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment, Renal Glycosuria, Glycaemic index & response to food consumed, Alimentary Hypoglycemia, Increased insulin response & sensitivity etc.

Glucose fasting, plasma as per ADA Guideline

NORMAL RANGE: <100 mg/dl

PRE-DIABETIC RANGE: 100-125 mg/dl (Impaired Fasting Glucose)

DIABETIC RANGE: = 126 mg/dl

Dr. Archa Rajagopal
Consultant Pathologist

Dr. Mohsin Kamaal
Consultant Pathologist

Page 2 Of 5



View Details



View Report

PERFORMED AT :

Agilus Diagnostics Ltd

Bhoomi Tower, 1st Floor, Hall No.1, Plot No.28 Sector 4, Kharghar

Navi Mumbai, 410210

Maharashtra, India

Tel : 9111591115, Fax :

CIN - U74899PB1995PLC045956



ULR No.775000015428602-0040

DIAGNOSTIC REPORT



PATIENT NAME : ANGEL JOSEPH

REF. DOCTOR : DR. DR MEENA

ANGEL JOSEPH
B-404, GOODWILL GARDEN, SEC 8, KHARGHAR
9870331316

ACCESSION NO : **0040YL001492**
PATIENT ID : ANGEF16030940
CLIENT PATIENT ID:
ABHA NO :

AGE/SEX : 16 Years Female
DRAWN : 07/12/2025 07:23:01
RECEIVED : 07/12/2025 07:27:51
REPORTED : 07/12/2025 12:13:54

Test Report Status	Final	Results	Biological Reference Interval	Units
--------------------	-------	---------	-------------------------------	-------

BIOCHEMISTRY - LIPID

LIPID PROFILE PLUS

LIPID PROFILE WITH CALCULATED LDL,SERUM

CHOLESTEROL, TOTAL	126	Desirable : < 200 Borderline : 200 - 239 High : > / = 240	mg/dL
METHOD : CHOLESTEROL OXIDASE, ESTERASE, PEROXIDASE			
TRIGLYCERIDES	80	Normal: < 150 Borderline high: 150 - 199 High: 200 - 499 Very High: > / = 500	mg/dL
METHOD : GPO - PAP			
HDL CHOLESTEROL	41	At Risk: < 40 Desirable: > or = 60	mg/dL
METHOD : CHOLESTEROL OXIDASE, ESTERASE, PEROXIDASE			
CHOLESTEROL LDL	69	Adult levels: Optimal < 100 Near optimal/above optimal: 100-129 Borderline high : 130-159 High : 160-189 Very high : = 190	mg/dL
METHOD : CHOLESTEROL OXIDASE, ESTERASE, PEROXIDASE			
NON HDL CHOLESTEROL	85	Desirable : < 130 Above Desirable : 130 -159 Borderline High : 160 - 189 High : 190 - 219 Very high : > / = 220	mg/dL
METHOD : CALCULATED PARAMETER			
VERY LOW DENSITY LIPOPROTEIN	16.0	< OR = 30.0	mg/dL
CHOL/HDL RATIO	3.1 Low	Low Risk : 3.3 - 4.4 Average Risk : 4.5 - 7.0 Moderate Risk : 7.1 - 11.0 High Risk : > 11.0	
LDL/HDL RATIO	1.7	0.5 - 3.0 Desirable/Low Risk 3.1 - 6.0 Borderline/Moderate Risk >6.0 High Risk	


Dr. Archa Rajagopal
Consultant Pathologist


Dr. Mohsin Kamaal
Consultant Pathologist

Page 3 Of 5



View Details



View Report

PERFORMED AT :

Agilus Diagnostics Ltd
Bhoomi Tower, 1st Floor, Hall No.1, Plot No.28 Sector 4, Kharghar
Navi Mumbai, 410210
Maharashtra, India
Tel : 9111591115, Fax :
CIN - U74899PB1995PLC045956



DIAGNOSTIC REPORT



PATIENT NAME : ANGEL JOSEPH

REF. DOCTOR : DR. DR MEENA

ANGEL JOSEPH
B-404, GOODWILL GARDEN, SEC 8, KHARGHAR
9870331316

ACCESSION NO : **0040YL001492**
PATIENT ID : ANGEF16030940
CLIENT PATIENT ID:
ABHA NO :

AGE/SEX : 16 Years Female
DRAWN : 07/12/2025 07:23:01
RECEIVED : 07/12/2025 07:27:51
REPORTED : 07/12/2025 12:13:54

Test Report Status	Final	Results	Biological Reference Interval	Units
--------------------	-------	---------	-------------------------------	-------

Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A. CAD with > 1 feature of high risk group B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease
Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

****End Of Report****

Please visit www.agilusdiagnostics.com for related Test Information for this accession

Dr. Archa Rajagopal
Consultant Pathologist

Dr. Mohsin Kamaal
Consultant Pathologist

Page 4 Of 5



View Details



View Report

PERFORMED AT :

Agilus Diagnostics Ltd
Bhoomi Tower, 1st Floor, Hall No.1, Plot No.28 Sector 4, Kharghar
Navi Mumbai, 410210
Maharashtra, India
Tel : 9111591115, Fax :
CIN - U74899PB1995PLC045956



ULR No.775000015428602-0040

DIAGNOSTIC REPORT



PATIENT NAME : ANGEL JOSEPH

REF. DOCTOR : DR. DR MEENA

ANGEL JOSEPH
B-404, GOODWILL GARDEN, SEC 8, KHARGHAR
9870331316

ACCESSION NO : **0040YL001492**
PATIENT ID : ANGEF16030940
CLIENT PATIENT ID:
ABHA NO :

AGE/SEX : 16 Years Female
DRAWN : 07/12/2025 07:23:01
RECEIVED : 07/12/2025 07:27:51
REPORTED : 07/12/2025 12:13:54

Test Report Status	Final	Results	Biological Reference Interval	Units
--------------------	-------	---------	-------------------------------	-------

CONDITIONS OF LABORATORY TESTING & REPORTING

1. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
2. All tests are performed and reported as per the turnaround time stated in the AGILUS Directory of Services.
3. Result delays could occur due to unforeseen circumstances such as non-availability of kits / equipment breakdown / natural calamities / technical downtime or any other unforeseen event.
4. A requested test might not be performed if:
 - i. Specimen received is insufficient or inappropriate
 - ii. Specimen quality is unsatisfactory
 - iii. Incorrect specimen type
 - iv. Discrepancy between identification on specimen container label and test requisition form
5. AGILUS Diagnostics confirms that all tests have been performed or assayed with highest quality standards, clinical safety & technical integrity.
6. Laboratory results should not be interpreted in isolation; it must be correlated with clinical information and be interpreted by registered medical practitioners only to determine final diagnosis.
7. Test results may vary based on time of collection, physiological condition of the patient, current medication or nutritional and dietary changes. Please consult your doctor or call us for any clarification.
8. Test results cannot be used for Medico legal purposes.
9. In case of queries please call customer care (91115 91115) within 48 hours of the report.

Agilus Diagnostics Ltd
Fortis Hospital, Sector 62, Phase VIII,
Mohali 160062

Dr. Archa Rajagopal
Consultant Pathologist

Dr. Mohsin Kamaal
Consultant Pathologist

Page 5 Of 5



View Details



View Report

PERFORMED AT :

Agilus Diagnostics Ltd
Bhoomi Tower, 1st Floor, Hall No.1, Plot No.28 Sector 4, Kharghar
Navi Mumbai, 410210
Maharashtra, India
Tel : 9111591115, Fax :
CIN - U74899PB1995PLC045956



ULR No.775000015428602-0040